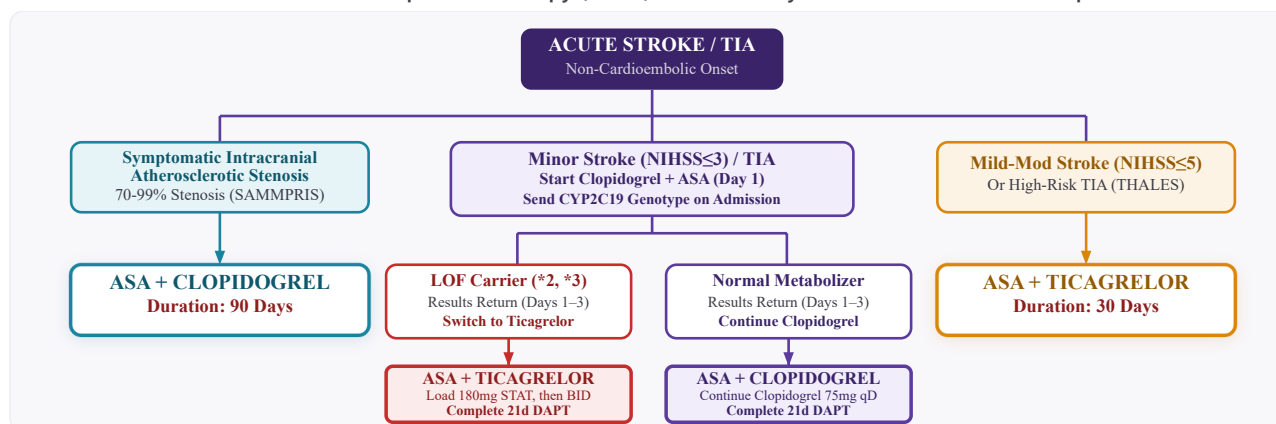


DAPT for Non-Cardioembolic Ischemic Stroke

Guideline-directed Dual Antiplatelet Therapy (DAPT) for secondary non-cardioembolic stroke prevention.



SCENARIO / TRIAL	TARGET POPULATION	LOADING DOSE (DAY 1)	DAPT DURATION	POST-DAPT
POINT Trial	NIHSS ≤ 3 or ABCD ² ≥ 4. **Within 12 hours** of onset.	Clopidogrel 600 mg + Aspirin 162–325 mg	Clopidogrel 75mg qD + Aspirin 81mg qD for 21 days	Aspirin 81mg
CHANCE / INSPIRES	CHANCE: NIHSS ≤ 3, ABCD ² ≥ 4 within 24h. INSPIRES: NIHSS ≤ 5, ABCD ² ≥ 4, ≥ 50% stenosis within 72h.	Clopidogrel 300 mg + Aspirin 75–300 mg	Clopidogrel 75mg qD + Aspirin 75-100mg for 21 days	Clopidogrel 75mg (to Day 90)
CHANCE-2 Trial	CYP2C19 LOF carrier (*2/*3) + Minor stroke/TIA. **Within 24h**.	Ticagrelor 180 mg + Aspirin 75–300 mg	Ticagrelor 90mg BID + Aspirin 75-100mg for 21 days	Ticagrelor 90mg BID (to Day 90)
THALES Trial	NIHSS ≤ 5 or high-risk TIA (ABCD ² ≥ 6 or symptomatic stenosis) within 24h.	Ticagrelor 180 mg + Aspirin 300–325 mg	Ticagrelor 90mg BID + Aspirin 75-100mg for 30 days	Aspirin 81mg
SAMMPRIS Trial	Severe symptomatic atherosclerotic stenosis (70-99%) of a major intracranial artery.	Aspirin 325 mg + Clopidogrel 75 mg (no load)	Clopidogrel 75mg qD + Aspirin for 90 days	Aspirin

CYP2C19 Genotyping & Clopidogrel Resistance

• CYP2C19 LOF alleles reduce clopidogrel activation. When rapid genotype results are available, LOF status can guide ticagrelor-vs-clopidogrel selection; CHANCE-2 evidence applies to LOF carriers rather than mandating universal testing.

Safety

- ****Bleeding vs. Benefit**:** For minor stroke/high-risk TIA, most DAPT benefit occurs in the first 21 days; extend longer only for selected trial-matched indications such as severe symptomatic intracranial stenosis.
- ****Post-Lytic / EVT Policy**:** After IV alteplase or TNK, avoid antithrombotics for the first 24h until follow-up imaging excludes hemorrhage. EVT alone is not a blanket DAPT contraindication; stenting/angioplasty plans and hemorrhage risk drive the decision.

POINT: Johnston SC et al. *N Engl J Med.* 2018;379:215-225. | **CHANCE:** Wang Y et al. *N Engl J Med.* 2013;369:11-19. | **CHANCE-2:** Wang Y et al. *N Engl J Med.* 2021;385:2520-2530. | **INSPIRES:** Gao Y et al. *N Engl J Med.* 2023;389:2413-2424. | **THALES:** Johnston SC et al. *N Engl J Med.* 2020;383:207-217. | **SAMMPRIS:** Chimowitz MI et al. *N Engl J Med.* 2011;365:993-1003.